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| 1. | What is shock defined as? | Inadequate tissue perfusion |
| 2. | aerobic produces energy in the prescense of | oxygen |
| 3. | anaerobic metabolism produces energy without | Oxygen, leading to the buildup of lactic acid. |
| 4. | When the body is in a shock state and is operating under an anaerobic metabolism, what condition is likely to develop due to lactic acid buildup? | Metabolic Acidosis. |
| 5. | What is the primary control center for the brain? | Medulla Oblongata |
| 6. | What are the nerve cells that keep our cardiovascular and respitory systems managed. | Chemoreceptors |
| 7. | Baroreceptors | Stretch receptors that balance the blood pressure |
| 8. | How long does it take for your sympathetic system to respond to shock? | Seconds |
| 9. | In compensatory shock, how long does your hormonal response take? | Several minutes |
| 10. | When can the body feel the peak effect of RAAS? | Up to three hours |
| 11. | effects of sympathetic nervous system (Fight of Flight) | |



	Epi/NorEpi release, an increase in HR/contractions, Vasoconstriction, Bronchodilation, Pupil dilation
12. what hormone will your pituitary gland release during compensated shock?	Antidiuretic hormone, containing water and increasing circulating volume
13. What does your renal perfusion sensor do during compensated shock?	Monitors glomerular filtration rate, and releases renin to increase GFR
14. Angiotensin I is a non vasoactive hormone that turns into	ACE Angiotensin Converting Enzyme
15. What is angiotensin II responsible for?	Increased BP and fluid volume by retaining sodium and water, and enabling vasoconstriction.
16. What are some signs of decompensated shock?	Noticeable change in mentation/AMS, Hypotension, Acidosis.
17. What are the four main types of shock?	Hypovolemic, Distributive, Cardiogenic, Obstructive
18. Hypovolemic shock is a result of a loss of...	Circulating volume
19. What are some common causes of NON hemorrhagic hypovolemic shock?	Burns, Dehydration, Loss of plasma
20. What are some common causes of hemorrhagic hypovolemic shock?	Blood loss, loss of clotting factors, decreased capacity to carry oxygen
21. What can lead to coagulopathy in shock?	Hypothermia, Acidosis, Liver failure, Hypokalemia, Hypocalcemia



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| 22. What four factors make up the trauma diamond? | Hypocalcemia, Hypothermia, Acidosis, Coagulopathy |
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| 23. How can we manage/treat hypovolemic shock? | Stopping bleeding, positioning, airway security, oxygenation/ventilation, warming, ALS, early blood resource |
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| 24. When the left ventricle loses about 40% of its normal output, this is considered... | Cardiogenic shock. |
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| 25. What categorizes distributive shock? | A loss of systemic vascular resistance, vasodilation, and increased vascular permeability. |
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| 26. Anaphylaxis falls under what category of shock? | Distributive |
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| 27. What hormone acts as your main vasodilator, causing GI upset and bronchoconstriction, and vascular permeability? | Histamine |
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| 28. Why is benadryl recommended for patients in anaphylactic shock? | It will bind histamine receptors |
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| 29. Septic shock is defined as... | A systemic infection resulting in widespread inflammatory mediator release |
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| 30. What are some S/S of septic shock? | Obvious/known sign of infection, High temperature, High HR, low SBP, AMS, low RR |
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| 31. What is the number 1 cause of sepsis in nursing homes? | UTI |
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	What are some hallmarks of neurogenic shock?	Hypotension with no cardiac compensation, flushed/warm skin below the site of injury.
33.	What are some causes of obstructive shock?	PE, Tension pneumothorax, Cardiac Tamponade
34.	What is virchows triad?	A warning sign for DVT's. Venous stasis, endothelial Injury, Hypercoagulability.
35.	How do we assess for JVD?	Sitting the patient upright
36.	What key physiological function is lost in neurogenic shock?	Loss of sympathetic functions responsible for maintaining vascular tone
37.	What defines obstructive shock?	Circulatory roadblock that prevents adequate blood flow despite normal heart function
38.	How can age influence shock?	Age affects how well patients compensate for and recover from shock
39.	How do children typically respond to shock?	Children compensate well initially but deteriorate quickly once decompensation begins